ELMSWORTH AGRICULTURAL — 2027 SYMPTOM CARD SOURCE PACKET

Source packet • internally indexed • issued 20 Aug 2026

Purpose and fixed production record

This packet is the sole approved source for the 2027 six-panel symptom-management quick-reference card used by facility nurses in the Elmsworth Agricultural hospice referral network. Preserve the current compact imperative voice and heading sequence. No supplement, QR code or live lookup is permitted.

Current edition structural pattern (source locator C-01)

Six panels in order: 1 ASSESS FIRST; 2 CONVERT OPIOID DOSE; 3 BREAKTHROUGH DOSE; 4 TERMINAL AGITATION; 5 MONITOR & ESCALATE; 6 DOCUMENT & HAND OFF. Each uses short labels and directive lines.

Panel limits and die-cut (source locator P-01)

Panel	Heading	Hard word limit	Finished die-cut / safe area
1	1 ASSESS FIRST	110	90 × 55 mm / 4 mm inset
2	2 CONVERT OPIOID DOSE	145	90 × 55 mm / 4 mm inset
3	3 BREAKTHROUGH DOSE	100	90 × 55 mm / 4 mm inset
4	4 TERMINAL AGITATION	135	90 × 55 mm / 4 mm inset
5	5 MONITOR & ESCALATE	120	90 × 55 mm / 4 mm inset
6	6 DOCUMENT & HAND OFF	135	90 × 55 mm / 4 mm inset

Each hard limit counts the printed panel heading and all clinical/footer copy; word totals use whitespace-delimited printed tokens. Proof geometry: one landscape 270 × 110 mm sheet; six 90 × 55 mm panels in a 3 × 2 grid. Dashed fold rules at x=90,180 mm and y=55 mm; keep all copy within the 4 mm inset. Permitted colors only: black text/rules and Elmsworth navy #17365D; white background.

Approved 2027 clinical and jurisdictional record

C-02 Opening assessment

Patient-specific authorization/order required before medication. Check allergy/sensitivity and last opioid dose, route, time. Symptom scale 0–10 with anchors 0 none / 10 worst. Reassess SC/IV 30 min and oral 60 min. Reversible contributors: pain, urinary retention, constipation, hypoxia, infection, medication effect. Comfort: reposition, reduce noise/light, mouth care, calm explanation, familiar person. Urgent red flags: sudden severe symptom, new focal deficit, suspected sepsis, uncontrolled bleeding, airway threat; do not delay escalation.

C-03 Conversion

Total scheduled plus PRN administered opioid over previous 24 h. Factors: oral morphine to SC morphine ×0.5; oral oxycodone to oral morphine ×1.5; transdermal fentanyl 25 microgram/hour to oral morphine 60 mg/day. Reduce new opioid 25–50% for incomplete cross-tolerance or frailty. Round DOWN to measurable 0.1 mL with listed strength. Calculation aid does not create/change order.

C-04 Transition

Legacy morphine 5 mg/mL may remain through 31 Mar 2027; end-state morphine 10 mg/mL from 1 Apr 2027. Confirm label concentration and calculate volume using that concentration. Never present two volumes without date/stock context. Recalculate before next dose if facility stock changes.

C-05 Breakthrough

1/6 of ordered total 24-hour scheduled opioid dose, same route, every 1 h PRN; minimum interval 1 h; not scheduled. Reassess SC/IV 30 min, oral 60 min. Record offered/administered/response/sedation. Contact hospice for ≥3 administered in 4 h or inadequate relief.

C-06 Agitation

Terminal agitation label applies to distressing restlessness/delirium with picking, calling out, fearful movements or altered attention in last days of life. Assess reversible contributors without delaying urgent escalation. Non-drug measures listed. Midazolam 2.5 mg SC PRN, repeat after 1 h, max 3 doses/4 h. If persists, hospice review and haloperidol 0.5 mg SC PRN q2h, max 2 mg/24 h, only if ordered. Reassess 30 min. Worsening, new rigidity or paradoxical response: hold and urgent call.

C-07 Monitoring/emergency

After doses observe symptom, consciousness, respiratory rate, airway, distress, response. Hold next opioid + call hospice for difficult to rouse, RR <8/min, severe myoclonus or repeated vomiting. Apnoea, unresponsiveness, cyanosis or airway threat: 999 then hospice. Naloxone if supplied/ordered is adjunct, never replacement. Cautions: renal/hepatic impairment, frailty, no prior opioid exposure require ordered reduction and prescriber review.

C-08 Documentation/contacts

Record assessment, order check, allergy, last dose, contributors, comfort, drug/concentration/dose/route/time/reason, response/reassessment, sedation/RR, PRN count, adverse effects, contacts/advice. Handoff at shift change and after escalation: symptom, last dose/time, stock concentration, PRN count, response, findings, plan. Contacts exact: Routine Tanneryside Hospice Desk 01632 960 410; Urgent Elmsworth Hospice Clinician 01632 960 411; Emergency 999 then Elmsworth Hospice Clinician.

Mandatory immutable wording (source locator L-01)

For use by trained facility nurses with a patient-specific order.

ELMSWORTH AGRICULTURAL | HOSPICE REFERRAL NETWORK | 2027 v1.0 | Effective 01 Jan 2027 | Approved: Tanneryside Advisory Clinical Governance | England jurisdiction.

Approved abbreviations (source locator C-09)

PRN = as needed; SC = subcutaneous; IV = intravenous; RR = respiratory rate. Units mg, mL, h and microgram/hour may be printed as shown.